

POTENTIAL KNOWN HAZARDS OR RISKS	RISK FACTORS	POSSIBLE CONTROLS
1. Manual tasks Our support and services may include manual tasks which include the handling of people including in their homes. Examples of common manual tasks include: • assisting with transferring, bathing and dressing clients • pushing wheelchairs • loading and unloading from vehicles • moving furniture • gardening and maintenance tasks • cleaning and other domestic tasks.	The risk of injury related to manual tasks increases when the physical aspect of the work requires: • repetitive or sustained force • high or sudden force • repetitive movement • sustained or awkward posture (i.e. prolonged holding of the worker's body part in one position) • exposure to vibration. Direct risk factors commonly seen include: • Forceful exertion – where the body has to generate significant force to perform the task (for example moving furniture). • Working posture – certain postures can strain body tissue, affect the amount of effort required to complete the task and quicken muscle fatigue. • Repetition/duration – performing a similar task over a prolonged period without a break, not allowing that part of the body to recover (for example mopping a large house). The contributory factors that cause these sources of risk include: • Work area design and layout such as: - furniture that promotes uncomfortable working postures because it is fixed at the wrong height or is non-adjustable (for example a low bed) - limited space or access to complete handling tasks – this will promote bending, twisting or being in an awkward or fixed posture (for example working in a cluttered bedroom) • Characteristics of the client when being assisted presents a major risk factor specific to people handling tasks:	Some possible controls for managing risks from manual tasks, including people handling, are: • eliminate (for example use a maxi taxi to transport clients in their wheelchairs instead of loading and unloading the wheelchairs into/out of a car boot) • provide: - mechanical aids or assistive devices (for example using hoists in a minimum lift approach for people handling) - smaller carry cases, boxes or cartons for taking items into the home - mobile and portable equipment supplied with dedicated trolleys or transfer/handling equipment • modify the workplace layout, process or equipment. Some modifications to the workplace layout and equipment may require negotiations with the building owner or client. Examples include: - installing grab rails and a shower hose in the shower - raising the height of the bed to minimise prolonged bending, (for example use purpose-built blocks under the legs of the bed to raise the height of the bed) - relocating furniture to allow enough space for the worker and the client to turn and carry out a task in a comfortable posture, or to move equipment such as a commode or wheelchair - storing equipment or objects within easy reach and storing heavy or frequently used items between knee and shoulder height - providing clear access through the home • develop a 'minimum lift' approach where all people handling tasks are evaluated and controlled so that workers are not handling all or most of a client's weight • train workers in safe work procedures including work methods and use of mechanical aids and equipment • ensure that the equipment provided is in good order and well maintained • plan work to alternate between heavy and light activities • ensure work/rest schedules are adequate for the work being done

	- size, weight, shape and dimensions - medical or physical condition of the client - communication capabilities (i.e. English as a second language or dysphasia following stroke) - cognitive functioning and ability to understand instructions or challenging client behaviours, including the client's ability or willingness to assist ● Systems of work – the way work is organised and how procedures are administered affects the level of risk, such as: - regular maintenance of equipment - appropriate staffing levels for the caseload (for example a number of highly dependent clients seen in one shift) - the need for a second person for certain transfers - extended work days or excessive work hours.	● ensure relevant information about the client is communicated to the Principal
2. Work related violence Workers can be exposed to work related violence. Risks may arise from the actions of clients, their Workers or others in the home or where the services are provided. If the potential for work related violence is not identified and managed properly, workers are at high risk of physical injury or psychological illness. Work related violence is any incident in which a person is abused, threatened or assaulted, in circumstances related to their work. Examples of work related violence include: ● biting, spitting, scratching, hitting, kicking ● throwing objects ● pushing, shoving, tripping, grabbing ● verbal threats ● armed robbery ● sexual assault ● attacking with knives, guns, clubs, or any type of weapon.	When are workers exposed to aggressive behaviour? Situations that may expose workers to the risk of work related violence include: ● providing services to clients who have challenging behaviours that may be related to a medical condition or intellectual impairment ● performing work alone or in isolated environments ● working in an environment where other people may pose a risk to workers' personal security (for example, client's family and friends). Potential risk factors to be considered when determining workers' exposure to work related violence are: ● Limited knowledge of client's behavioural triggers ● type of challenging behaviour workers may be exposed to (for example verbal abuse vs physical abuse) ● frequency and severity of exposure to challenging behaviour; look at incident or hazard reports ● layout of the workplace for example, inability of the worker to remove themselves from the area if required.	Controls which can be used to prevent or minimise the risks from work related violence include: ● working with clients to identify behavioural expectations surrounding service provision and the consequences to service provision if these are not met (check that the client clearly understands) ● redesigning or refurbishing existing environments, so they are more secure (for example provide a 'safe room', remove potential weapons from the environment, or install a physical barrier in cars between the driver and the client) ● ensuring relevant information about the client is communicated to the Principal ● reviewing the requirement for working alone and providing additional service providers where required ● providing reliable communication devices to maintain contact between the employer and the worker ● providing personal duress alarms and training staff in their use ● ensure workers have access to well-maintained motor vehicles so they do not break down in unsafe locations or times ● developing and implementing procedures for workers and managers setting out preventative strategies and the steps to be taken if an incident occurs ● avoiding the need for workers to carry money or valuables:

	● need for workers to carry money or medication ● the time it would take to have a supervisor/shift manager attend the house to support the worker ● change of service with no review of existing controls put in place to minimise the original risk.	- organise direct debit system for collecting co-payments - provide a suitable container for carrying money or medication that is not easily identifiable - discourage workers from wearing jewellery and carrying large amounts of cash ● refusing or modifying services until risks are eliminated or minimised (the referring agency and other service providers should be advised of this situation) ● authorise workers to discontinue services if they believe their personal safety is at risk ● providing services in a more secure environment (for example community centres) where possible and appropriate ● providing training for workers on: - reporting all hazards including abuse and threats - dealing with challenging client behaviour by ensuring workers understand the client's care plan - de-escalation and avoidance strategies - seeking assistance before the situation becomes critical. - the organisation's policies and procedures relating to aggressive behaviour - review of existing controls put in place to minimise the risk ● providing support personnel as necessary ● providing counselling services for workers as required.
3. Work Related Stress Work related stress describes the physical, mental and emotional reactions of workers who perceive that their work demands exceed their abilities or their resources (such as time, help / support) to do the work. Stress responses occur when the worker perceives they are not coping in situations where it is important to them that they do. How to determine the source of occupational stress The source of work related stress can be determined by evaluating:	Stressors or risk factors for occupational stress may include: ● work demands (i.e. workloads or excessive demands from employers, clients or others at the workplace (physical, emotional, and cognitive) ● lack of role clarity (i.e. poorly defined job roles and reporting structures) ● low control of what work tasks are done and how they are performed ● poor support from managers, supervisors, and/or peers (this may include working alone or in an isolated environment) ● poorly managed relationships (i.e. conflict or work relationship)	Controls that can be used to manage work related stress risks include: ● regularly review staffing levels to ensure appropriate staffing skills mix and numbers ● provide clearly defined job descriptions, policies and procedures ● ensure supervisors and managers have the skills required to manage their work team (i.e. able to support workers whilst managing their performance and adherence to policies and procedures. ● review organisational and performance management systems ● have policies and procedures for managing conflict and workplace bullying ● provide staff training and strategies on how to manage workloads, resolve

● productivity levels ● rates of absenteeism ● separation rates/turnover ● exit interviews ● staff engagement/morale ● client feedback ● peak/seasonal demands ● incident reports and data trends.	problems with supervisors and/or colleagues) ● exposure to emotionally distressing situation or incidents involving a threat to wellbeing (for example physical violence or the threat of physical violence with or without a weapon) ● poorly managed change, low levels of recognition and reward ● emotional attachment to a terminally ill client. To conduct a risk assessment for work related stress, the risk factors listed above should be considered by: ● acknowledging and understanding worker complaints and where necessary investigating ● observing interactions between workers, and between workers and clients ● having one-on-one discussions with workers ● conducting focus groups or worker surveys with tools such as the psychosocial risk assessment process People at Work.	conflict, job rotation, maintaining a balanced relationship and appropriate boundaries with client ● provide counselling services for staff where applicable ● refuse or modify services to the client if an environment is too high risk.
4. Remote or isolated work This is work that is undertaken when isolated from the assistance of other people because of the location, time or nature of the work being done. Assistance from other people includes rescue, medical assistance and emergency services.	A worker may be isolated even if other people may be close by, for example, a Worker working by themselves overnight. In other cases, a worker may be far away from populated areas, for example, on a farm. In some situations, a worker may be alone for a short time. In other situations, the worker may be on their own for long periods of time, for example driving in remote locations.	Controls that can be used to manage remote or isolated work include ensuring effective communication with the worker carrying out remote or isolated work.
5. Biological hazards Biological hazards expose workers and clients to infection risks. Good infection prevention and control practices will protect workers and clients from acquiring healthcare associated infections. Some infectious diseases such as rubella (i.e. German measles), cytomegalovirus and varicella (chicken pox) may pose additional risks to pregnant workers with the potential for adverse pregnancy outcomes. Infection from exposure to a biological hazard can	Workers may be exposed to infectious diseases through activities such as: ● health and personal care of clients ● contact with a client's blood and body substances ● handling contaminated items and equipment ● household cleaning, including cleaning spills of blood and body substances	Some of the controls that can be used to manage infectious disease risks include: ● ensuring workers adopt standard precautions for the care and treatment of all clients and when handling all blood and body substances, non-intact skin and mucous membranes. This includes: - personal hygiene practices, particularly hand hygiene, including covering non-intact skin (for example cuts, dermatitis) with a water-resistant dressing - correct use of personal protective equipment, which may include gloves, gowns, plastic aprons, surgical masks, safety eyewear and face shields

cause serious illness. Any infection to which the carrying out of work is a significant contributing factor must be notified to relevant external bodies. This includes any infection that is reliably attributable to carrying out work that involves providing treatment or care to a person or involves contact with human blood or body substances.	● handling soiled laundry ● handling and disposing of clinical waste including sharps ● unsafe food handling and storage practices ● contact with a client's animals and animal excreta.	- safe handling and disposal of sharps, and using safety engineered medical devices such as retractable needles where appropriate - maintain a clean work environment and manage spills of blood and body substances - hygienically handle and launder soiled linen - handle and dispose of all waste appropriately, including clinical waste - correctly process reusable equipment and instruments ● ensuring that workers adopt transmission-based precautions for clients known or suspected to be infected or colonised with infectious agents that may not be contained by standard precautions alone. These are additional work practices needed to contain infection risks and should be tailored to the particular infectious agent involved and its mode of transmission. This may include wearing specific personal protective equipment like a respirator and using dedicated equipment for the personal and health care of the client ● having protocols for managing accidental exposure to blood and body substances, sharps injuries and other infectious disease exposures including first aid, medical referral and access to post-exposure prophylaxis (chemoprophylaxis) where indicated ● providing information, instruction, training and supervision in infection control practices ● providing workers with hand hygiene facilities (for example alcohol-based hand rub) where hand washing facilities are not readily available in a client's home ● implementing an occupational immunisation program in accordance with the current edition of the Australian Immunisation Handbook (Department of Health, Australian Government) ● using appropriate work placements and work restrictions (for example do not assign a nonimmune worker to care for a client with a known vaccine-preventable disease such as varicella) ● providing adequate supplies of personal protective equipment in a range of sizes and instruct workers in the correct selection and use of the equipment ● securing aggressive dogs before the worker enters the workplace to prevent animal bites and ensure that animal excreta is hygienically cleaned.
6. COVID-19 In line with expert health advice about what the community must do to prevent the spread of		Protect workers from the risk of exposure to COVID-19 by, for example: ● considering working from home arrangements

COVID-19, Australian governments have issued enforceable government directions which set out requirements and restrictions for businesses in each state and territory. These public health directions include matters such as:

- limits on public gatherings
- physical distancing and density requirements (such as 1.5 metre distancing and 4 square metres of space per person)
- limits and restrictions on operations, including types of businesses or facilities remaining closed and limitations on activities
- hygiene, cleaning and signage requirements
- risk management requirements such as COVIDSafe plans (or similar)
- keeping records of contact details for the purposes of contact tracing, and
- penalties and compliance.

Not all public health directions will be the same in each state and territory or for each business, so it is important you review the directions which apply to your workplace.

Remember, even if these public health directions don't apply to your workplace, you must still do all you reasonably can to meet your work health and safety duties and limit the spread of COVID-19.

- requiring workers to practice physical distancing
- requiring workers to practice good hygiene (e.g., through workplace policies and ensuring access to adequate and well stocked hygiene facilities)
- requiring workers to stay home when sick, and
- cleaning the workplace regularly and thoroughly.

Protect others from the risk of exposure to COVID-19 by, for example:

- requiring them to practice physical distancing, including through contactless deliveries and payments
- requiring them to practice good hygiene, and
- requiring others to stay away from the workplace, unless essential, e.g., such as family, friends and visitors.

You must maintain your workplace to ensure the work environment does not put workers and others at risk of contracting COVID-19.

Maintain a safe work environment by, for example:

- cleaning the workplace regularly and thoroughly
- restructuring the layout of the workplace to allow for physical distancing, and
- limiting the number of people in the workplace at any given time.

You must also provide adequate facilities in your workplace to protect your workers from contracting COVID-19. Facilities that are required include:

- washroom facilities including adequate supply of soap, water and paper towel
- hand sanitiser, where it is not possible for workers to wash their hands, and
- staff rooms that are regularly cleaned and allow for physical distancing.

Provide workers with regular breaks to use these facilities, particularly to allow workers to wash their hands.

7. Latex Allergy Some people may develop allergies to latex products.	Latex products can cause: ● irritant contact dermatitis This non-allergic condition is the most common reaction to latex products and is caused by: - skin irritation from the accumulation of moisture, sweat, soaps and detergents on the skin - incomplete hand washing and drying - prolonged glove use - corn starch which is added to some latex gloves ● dry, itchy skin, usually on the hands which resolves once contact with the latex product is discontinued ● allergic contact dermatitis (also known as delayed hypersensitivity reaction or Type IV) is caused by an allergy to chemicals added during the manufacturing of latex gloves (for example thiurams and carbamates). It causes a rash and blisters on the hands, usually occurring several hours after contact. Repeated exposure may cause the skin condition to extend beyond the area of contact with the latex product ● Latex sensitivity (also known as latex allergy, immediate hypersensitivity reaction or Type I) is caused by an allergy to latex proteins and is a more serious condition. Symptoms usually occur soon after exposure and include: - a local or generalised skin rash - hives - itchy eyes - runny nose - wheezing - anaphylactic shock, which is a life-threatening emergency	Some controls for managing latex allergy hazards include: ● eliminate non-essential use of latex gloves, (for example provide workers with non-latex gloves for activities that do not involve contact with blood and body substances such as routine housekeeping or food preparation) ● provide low protein, powder-free latex gloves or latex-free gloves such as vinyl or nitrile gloves ● provide workers with information on latex allergy and safe work practices such as: - instruct workers to wash their hands with soap and water after removing latex gloves to remove natural rubber latex proteins from the skin - remind workers to not use oil-based creams or lotions with latex gloves, as these can cause the gloves to deteriorate - instruct workers to report health problems from the use of latex gloves, and ensure that they seek medical attention - identifying clients who may have a latex allergy and ensuring that health and personal care is provided to these clients in a latex-safe environment.
8. Hazardous substances/chemicals There are a number of chemicals used in the services and supports we provide, particularly for cleaning, laundry and gardening tasks.	For those chemicals that are classified as hazardous, the health effects are based on the extent of exposure that a person experiences, which is affected by many factors such as: ● the frequency of use (for example once a week versus several times a day) ● quantities being handled (for example 10 L drum of product versus small packs) ● concentration/strength of the chemical (for example diluted versus undiluted)	Ensure that there are systems in place to control hazardous chemical products used in clients' homes. This may include: ● providing the chemicals being used, or providing guidelines to clients about choosing appropriate chemicals. Wherever possible, non-hazardous chemical products should be sought that are fit-for-purpose. ● any suspected hazardous chemical without a label must not be used. ● decanting chemicals should be avoided where possible. If decanting cannot be avoided, the entire contents of the container holding the decantation should be used or emptied and then cleaned. If the entire contents of the container are not used immediately, a label must be fixed to the container

	● mechanism of application (for example use of sprays/aerosols versus pastes) ● working in areas with poor ventilation like shower alcoves, ovens or small gardening sheds compared to open areas with good airflows to disperse an air-borne chemical.	stating the chemical's product name and risk and safety phrases. Unlabelled containers that are used completely should be discarded immediately after use ● eliminate the use of hazardous chemicals wherever possible. Assess if the task is essential, to establish if it can be eliminated ● use the substance in a different way that prevents or minimises the risk from exposure to the substance ● substitute a hazardous chemical with a less hazardous one, which is better suited for domestic use ● use an exhaust fan or open windows for adequate ventilation while working with the substance ● appropriate selection, use and maintenance of personal protective equipment (PPE) and clothing and provision of these ● first-aid measures and emergency response actions ● safe storage of chemical containers.
9. Electrical Safety Workers may have to use the client's electrical installation and electrical equipment (i.e. power points, lights, extension leads etc.).		When the electrical equipment does not belong to us, Workers should: ● visually inspect the electrical installation to satisfy themselves as to the electrical safety of the installation, paying particular attention to details such as damaged or missing parts and burning or discolouration of the electrical fittings in the installation (for example damaged light switch or cracked power point) ● avoid using the client's electrical equipment (i.e. electric kettles, vacuum cleaners, extension leads etc.) if possible. If the client's equipment is to be used, then it should be visually inspected before use, with attention to damaged insulation, missing parts (for example frayed leads, faulty switches, exposed wiring). Although many residential premises have their socket outlet circuits fitted with a safety switch, there is no regulatory testing requirement for safety switches in a domestic residence. In such circumstances, the employer or self-employed person should use their own compliant portable safety switch. Employers and self-employed persons should ensure that workers are trained in the safe use of electrical equipment. Training should include how to:

		● conduct a visual inspection ● carry out the push button test of a safety switch. Extension leads should be: ● located where they are not likely to be damaged and do not present a trip hazard when in use ● fully extended prior to using if fitted to a coiling device or reel. Electrical equipment should be used and stored in a manner which does not damage the electrical fittings (for example don't overstretch extension cords). It should not be exposed to harsh or damaging environments (for example chemicals or water) unless the equipment is specifically designed for use in these environments. Extra low voltage or battery-powered equipment may be an alternative in these environments. Use of double adaptors and 'piggy back' type plugs should be discouraged and alternatives such as power boards used. Keeping records of electrical equipment and its testing requirements will help ensure adequate testing is carried out when required. Once issues are identified, repairs should be carried out promptly by an authorised repair agency or alternatively supply other equipment which is in good order.
10. Slips, trips and falls Workers, clients and their families may be exposed to slip and trip hazards inside and outside the house. Slips usually occur when there is a loss of grip between the shoe and floor (i.e. when there is a contaminant between the shoe and the floor). Trips occur when a person's foot hits a low obstacle in the person's path, causing a loss of balance. It is often due to an obstacle that is not easily seen or noticed.	Common risk factors that contribute to slips and trips are: ● contaminants – can be anything that ends up on a floor. It could be wet (for example water or oil), or dry (for example talcum powder or plastic bags) ● slippery floor surfaces, especially in areas which may become wet or contaminated (for example bathrooms and toilets) ● obstacles and other trip hazards – trips most often occur because of uneven flooring or cluttered walkways with low obstacles which are not easily seen or noticed. Common examples of low obstacles include: - electrical leads - uneven edges to flooring - loose mats or carpet tiles - changes of floor surface levels.	Some simple and cost-effective measures that can reduce or prevent the number and severity of slips, trips and falls injuries include: ● prevent floor contaminants and attending to spills immediately ● minimise walking on recently cleaned floors until fully dried ● clean floors properly with the right amount and type of cleaning product used so that the floor does not become too slippery. ● clear growth (for example moss and slime) and leaf litter from outdoor pathways. ● put anti-skid tape on external steps to improve surfaces as a short- to-medium term measure. ● good housekeeping practices.

		● ensure the floor surface is in good order such as being free from: - holes - uneven surfaces - curled up linoleum - carpet edges ● avoid changes in floor surface level, or if this is not possible, highlight these changes (for example on the edge of the step in a split level home) ● provide adequate storage facilities. ● ensure lighting is adequate to see the area clearly without glare or shadowing to highlight potential slip or trip hazards. ● check and replace light bulbs to appropriate wattage. ● ensure workers can maintain their balance when: - carrying a load - workers should have full view of where they need to travel and should also have a free hand to hold onto a rail when walking down steps - using a ladder - ensure the correct ladder is available for the job and at least three points of contact are maintained at all times to maintain stability and balance. ● Choose footwear that is: - suitable for the type of work and work environment - comfortable with an adequate non-slip sole and appropriate tread pattern - checked regularly to ensure treads are not worn away or clogged with contaminants - designed to provide support and stability when worn.
11. Driving risks for workers Driving motor vehicles is a significant part of a worker's day when providing services in people's homes.	The hazards when driving and transporting clients may include: ● poor weather or road conditions ● fatigue and driving when tired ● rushing due to tight timeframes and scheduling of work ● distractions within the vehicle (i.e. mobile phones, client behaviour) ● safely operating unfamiliar vehicles	Possible controls that should be considered to reduce motor accidents include: ● selecting and purchasing safe vehicles (i.e. appropriate safety accessories / extras, airbags, bull bars, ABS brakes, tinted windows) ● ensuring vehicles are appropriately insured ● maintaining a safe vehicle by: - use an accredited mechanic for regular maintenance in accordance with vehicle requirements - daily motor vehicle checks by the driver (for example operational lights, condition of tyres)

	● managing client behaviour when travelling in vehicle ● poorly maintained vehicles (for example tyres, brakes, lights) ● speeding and or not following road rules ● changes to usual route/s taken (i.e. roadworks) and traffic delays ● unrestrained equipment in the vehicle ● remote and isolated work.	- ensure drivers report defective vehicles and all incidents resulting in injuries or damage ● maintaining a system of: - recording and monitoring incidents - monitoring each driver and vehicle (for example accident and service records) - develop and implement a safe driving policy for the organisation, including road rules such as wearing seat belts, mobile phone use, and safe use of any other in-vehicle connectivity ● providing workers with information and instruction on how to operate the allocated vehicle safely ● encouraging workers not to drive on roads if visibility or the road conditions are poor ● ensuring drivers are competent and fit to drive by periodically confirming they are appropriately licensed, reporting any driving offenses which may affect the ability to drive at work, and including this requirement in position descriptions ● notifying them of safe driving policy and the requirement to advise of any medications or medical conditions which may impair their ability to drive ● making information on road rules and defensive driving training available to all workers ● reviewing workload and timeframes to ensure adequate time to complete work and travel between clients ● reducing driving times by: - planning journeys and sharing driving if required - grouping clients together - using alternatives such as public transport - providing workers with up-to-date information on changes to routes due to road closures and road works ● securing all equipment for transport (for example in the car boot or behind a cargo barrier) ● ensuring staff transporting dangerous, awkward goods, or people with wheelchairs, are appropriately trained for the task, have access to appropriate mechanical aids, and/or have appropriate assistance at start
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12. Fatigue Fatigue is mental or physical exhaustion which stops a person's ability to perform work safely and effectively. Fatigue can adversely affect safety in the workplace. Fatigue can be caused by factors which may be work related, non-work related or a combination of both and can accumulate over time.	Potential causes of fatigue include: ● mentally and physically demanding work ● long periods of time awake (i.e. long hours of work extended by long commuting times) ● inadequate amount or quality of sleep (i.e. when on call) ● regular work at night ● work requirements or systems of reward (i.e. pay, recognition or promotion) that provide incentives to work longer and harder than maybe safe ● poor work scheduling and planning.	Ensure workers aren't required to work extended hours by: ● arranging sufficient cover for workers who are on annual or sick leave ● planning for necessary overtime so workers can schedule their activities around it ● ensuring shift work rosters provide for adequate sleep in a 24 hour and even day period. ● avoiding working arrangements that provide incentives to work excessive hours. ● including adequate rest breaks between shifts to allow employees enough recovery time (i.e. time needed for travelling, eating, sleeping and socialising).